

Gastroesophageal Cancer: Recent Changes and What's on the Horizon

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A special ASCO Post Evening News discussion with Drs. Daniela Molena and Samuel Cytryn of Memorial Sloan Kettering Cancer Center. It covers landmark trial data, multidisciplinary care, surgery, and what's next for personalized treatment in esophagogastric adenocarcinoma, a cancer where the esophagus meets the stomach.

Meet the Experts

Two MSK leaders in esophagogastric cancer care.



Daniela Molena,
MD

Thoracic surgeon and Director of the Esophageal Surgery Program at MSKCC. She focuses on optimizing patients for surgery within multimodal (multiple treatment) plans and supports systemic perioperative (around surgery) therapy.



Samuel Cytryn, MD

GI medical oncologist (cancer doctor specializing in the digestive tract) at MSKCC. He integrates immunotherapy (treatment that helps the immune system attack cancer) into curative-intent regimens and times treatment with resection (surgery to remove the tumor).

ESOPEC and MATTERHORN: Two Home Runs

ESOPEC

FLOT (4-drug chemo) beat CROSS chemoradiation (chemo plus radiation before surgery) in resectable esophageal adenocarcinoma.

MATTERHORN

Durvalumab (immune drug) was added to perioperative FLOT. Early data showed no added surgical risk.

FLOT4

FLOT beat ECF/ECX in gastroesophageal adenocarcinoma. Median overall survival improved from 35 to 50 months.

SURGICAL PERSPECTIVE

The Surgical Perspective on MATTERHORN

Dr. Molena

*Thoracic Surgeon,
MSKCC (Memorial
Sloan Kettering
Cancer Center)*

- MATTERHORN tracked R0 resection and postoperative complications.
- Outcomes were comparable between arms, with no signal that durvalumab hurt resectability or recovery.
- Durvalumab was added safely to FLOT, supporting perioperative use.

Systemic Therapy Catches Up

- Surgery alone could not prevent distant recurrence in many esophagogastric cancers.
- Esophagogastric cancer is now treated as a systemic disease, with perioperative therapy targeting micrometastases.
- ESOPEC showed FLOT lowered distant recurrence versus chemoradiation.



Better neoadjuvant therapy still has to preserve curative surgery.

The Power of the Team Approach

Team coordination keeps treatment effective and surgery on track.

Tailor intensity

Reassess each cycle and adjust FLOT plus durvalumab based on response and toxicity.

Stay flexible

Reduce treatment if neuropathy or fatigue threatens surgical fitness.

Decide together

Surgeon and oncologist must agree on the best time for surgery.

Minimally Invasive Surgery and Nutrition

Minimally Invasive Surgery

At high-volume centers like MSKCC, robotic and laparoscopic esophagectomy has largely replaced open surgery for many patients.

It means fewer lung complications and a shorter hospital stay.

Nutrition Management

After esophagectomy, patients fill up quickly and may not meet calorie needs by mouth alone.

A jejunostomy feeding tube can help support strength through chemotherapy.



Restart chemotherapy as soon as it is safe, ideally within 8 to 12 weeks.

Looking Ahead



ctDNA Testing

ctDNA may detect residual disease after surgery, but current tests can miss cancer.



Nonoperative Management

Reliable tests could help some complete responders avoid esophagectomy.



Targeted Agents

Claudin 18.2 and HER2 are promising targets in gastroesophageal cancer.

Connecting to the Science

01

Identify the gap

Older regimens kept 5-year survival below 25 percent.

02

Review trials

Compare FLOT4, ESOPEC, and MATTERHORN for endpoints, safety, and populations.

03

Form questions

Ask how ctDNA, pCR, and nonoperative management affect outcomes.

04

Share insights

Trace the shift toward immunotherapy-enhanced perioperative regimens.

Key Takeaways: What Changed, What Was Solved, and What Remains

What Changed

For more than a decade, CROSS (a standard chemo-radiation plan) was the main approach for esophageal adenocarcinoma. ESOPEC shifted that standard by showing FLOT produced better survival, and MATTERHORN added durvalumab to perioperative FLOT without hurting surgical safety.

What Was Solved

FLOT is now the clear perioperative chemotherapy backbone, with a validated path to add immunotherapy. Better adjuvant therapy completion is also being helped by minimally invasive surgery and proactive nutrition support.

What Remains

Patient selection is still the biggest gap. Clinicians still cannot reliably identify who has a true pathologic complete response and might safely avoid esophagectomy, and ctDNA still needs prospective validation.



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